



Working with men in the context of distressed and disrupted intimate partner relationships: A qualitative study

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ABSTRACT

Objective: To describe key considerations for working with men experiencing distressed and disrupted intimate partner relationships.

Methods: Individual Zoom interviews were conducted with help-seeking men (n = 25) who had experienced an intimate partnership break-up, and health service providers (n = 30) working with men in the relationships space. Interpretive Description methodology was used to generate considerations for working with men in distressed and disrupted relationships.

Results: Three thematic findings were inductively derived; 1) *A whole life approach for deconstructing relationships*, wherein men engaged in discussions about their broader experiences and circumstances within the context of intimate partnerships; 2) *Affirming men's relationship emotions and vulnerabilities as normative and changeable*, comprising coaching for embodying transformative masculinities; and 3) *Tangible 'to do's' in and after a relationship*, outlining men's present and prospective self-work with action-oriented strategies.

Conclusion: Strategies tailored to men's receptivity and needs can increase connection with professional services and providers to bolster the mental health of men in and after disrupted intimate partner relationships.

Practice implications: With men increasingly accessing professional mental health services, the present study offers key considerations and recommendations regarding assessment, communication, and treatment for health service providers working with men in the relationships space.

1. Introduction

Healthy intimate partner relationships are linked to better overall health and well-being for men [1–3]. However, when relationships break down, men's risk for mental illness significantly increases [4]. Men who are recently separated (<2 years) or in poor quality relationships have greater suicide risk [5], with separated men being eight times more likely to die by suicide than separated women [6]. Several factors may underpin this disparity, including men's alignments to idealized masculinities that norm self-reliance, stoicism and strength for independently overcoming challenges [7]. Additionally, men report

considerable shame and grief associated with the loss of masculine status related to socialised ideals of being a protector, provider, and/or family man, further undermining their well-being [8]. Concealing, internalizing, and/or self-medicating associated vulnerabilities can further elevate men's risk for psychological distress and self-harm [9–11].

Despite longstanding commentaries that depict men as ubiquitously reticent to seek professional help, there is emergent evidence of men's diverse reasons and pathways in accessing mental health care [12–14]. In the wake of a relationship break-up, men may seek informal help online, from friends and family, community and peer-based supports,

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and/or professional care services [12]. As increasing numbers of men seek professional mental health help [10], research indicates relationship and family issues are amongst the most common catalysts for accessing services, particularly among men aged 30-years or older [13, 14]. While some men access counselling and therapy services directly, general practitioners can be the initial contact and conduit [13]. Oliffe et al. [12] suggest that men engage professional mental health care after an intimate partner break-up to cope, build self-management skills, and/or self-improve for future relationships. Factors that impact men's mental health and help-seeking preferences include complex individual (e.g., age, sexual orientation, adherence to masculine norms) and relationship factors (e.g., recency, duration, child custody).

Most empirical work into services for men in disrupted relationships has focused on behaviour change for men who perpetrate intimate partner or domestic violence [4]. Investigations into men's disrupted relationships from a broader perspective (i.e., beyond the context of violence) are relatively absent from the literature with extant research focused on men's mental health and suicide risk (e.g., [8,15]), help-seeking patterns and preferences (e.g., [12,16]), and emotions in and after an intimate partner relationship (e.g., [17,18]). It follows that research into health service providers' perspectives and help-seeking men who have experienced disrupted intimate partner relationships is needed to guide gender-transformative services that lobby men to critically examine, challenge and reshape gendered norms, practices and structures [19,20]. The current study purposefully reports the experiences of help-seeking men and health service providers to describe key considerations for working with men who experience distressed and disrupted intimate partner relationships.

2. Methods

2.1. Participants and recruitment

Participants comprised thirty Australian and Canadian-based health service providers and twenty-five help-seeking men. Health service providers included counsellors, social workers, and psychologists to reflect men's diverse help-seeking pathways, and eligibility criteria was based on their experience working in the relationships space with men. Men (18+ years) were eligible if they experienced a break-up and sought professional help in and/or after the relationship. Recruitment occurred online via Twitter, Reddit and Facebook inviting potential participants to contact the project manager. Relevant organizations were also identified and health service providers were contacted directly by email. Eligible participants were provided a link to an online consent form and demographics questionnaire, and a mutually convenient time was scheduled to conduct an individual Zoom interview [21]. Ethics approval was obtained from the University of British Columbia (H20-1868). Participants received a \$100 e-gift card to acknowledge their time and contribution to the study.

2.2. Data collection

Participants completed a brief descriptive survey to characterize the sample (e.g., age, sexuality, depressive symptoms (PHQ-9 [22])). In-depth semi-structured individual interviews, lasting approximately 65 min with health service providers and 75 min with help-seeking men (range 60–90 min), were conducted by 4 researchers based in Canada. Interview guides for help-seeking men and service providers were developed based on study objectives (Appendix A and B). Interview questions with help-seeking men included, "When should men access help for building better relationships?" and "Which aspects of the help you received were most valuable?" Interviews with health service providers focused on their experiences working with men, and questions included, "How do you support men to build relationships and navigate breakdowns?" and "What works best for men in terms of accessing and sustaining engagement with services?" Interviews were transcribed

verbatim, accuracy checked, and cleaned to remove potentially identifying details.

2.3. Data analysis

Interpretive description was used to address the research question, *what are key considerations for working with men who experience distressed and disrupted intimate partner relationships?* Interpretive description guides the exploration of characteristics, patterns and structures to generate knowledge focused on clinical practice and applied health research [23]. Constant comparative analysis and data triangulation were used to inductively derive empirical insights drawn from the experiences and perspectives of help-seeking men and health service providers. Data were inductively coded by two researchers to ten descriptive labels, including 'life impacts', 'assessing risk', 'effective communication', and 'treatment preferences' to advance preliminary interpretations. Data in each code were read and re-read and analyses discussed by the research team to build consensus and account for variations on the patterns. Some codes were subsumed and combined to more broadly but contextually aggregate the data and formally compare the help-seeking men and health service provider perspectives to distil key considerations and recommendations. Final themes, including key considerations and recommendations for clinical practice, were discussed and refined throughout the analyses and writing of the current article by the research team. Patterns and practices were identified for which explanatory gendered notes and therapeutic frames were developed. In line with the constructivist orientation of interpretive description [24], we advanced the gender analyses and application of the study findings with Connell's masculinities framework [25]. Here-with, masculinities can be understood as performative and malleable, and therefore responsive to tailored services [25] and transformative strength-based healthy masculinities [26].

3. Results

3.1. Participants

Thirty health service providers and 25 help-seeking men were individually interviewed to describe key considerations for working with men experiencing distressed and disrupted intimate partner relationships. Health service providers identified as heterosexual ($n = 30$; 100 %) men ($n = 27$; 90 %), ranged in age from 30 to 69 years ($M=49.1 \pm 10.1$), and had 1.5–35 years of experience working with men ($M=12.3 \pm 11.9$). Help-seeking men self-identified as heterosexual ($n = 21$; 84 %), ranged in age from 27 to 70 years ($M=41.2 \pm 10.7$), and reported discussing their relationship breakdown with a range of health care professionals including psychologists, psychiatrists, counsellors, family doctors, and social workers. Relationship duration ranged from 9 months to 28 years ($M=9.3 \pm 6.8$) and almost three quarters of the men were currently single and previously separated or divorced at the time of the interview ($n = 18$; 72 %). Based on the PHQ-9 [22], over half the help-seeking men indicated mild to severe depression symptoms (score ≥ 10) over the past two weeks ($n = 16$; 64 %). Six (24 %) participants indicated suicidal ideation in the past 2-weeks. Participant characteristics for health service providers and help-seeking men are presented in Table 1 and Table 2, respectively.

3.2. Thematic findings

Findings revealed the influence of intimate partner relationships on men's mental health and the role of health service providers working with men to address related challenges. Detailed are three themes: 1) A whole life approach for deconstructing relationships, 2) Affirming men's relationship emotions and vulnerabilities as normative and changeable, and 3) Tangible 'to do's' in and after a relationship. Table 3 provides a summary of themes with recommendations for clinical practice.

Table 1
Characteristics of health service providers (N = 30).

Characteristics	n (%)
Country	
Canada	16 (53)
Australia	14 (47)
Gender	
Man	27 (90)
Woman	2 (7)
Non-binary	1 (3)
Age (years)	
30–39	8 (27)
40–49	6 (20)
50–59	10 (33)
60–69	6 (20)
Sexuality	
Heterosexual	30 (100)
Education	
Some or all High School	1 (3)
Diploma or Certificate	4 (13)
Some College	2 (7)
Bachelor's Degree	7 (23)
Post Graduate Degree	16 (53)
Profession	
Coach	2 (7)
Counsellor	10 (33)
Educator	2 (7)
Group Facilitator	2 (7)
Researcher	2 (7)
Psychologist	2 (7)
Psychotherapist	2 (7)
Social Worker	2 (7)
Other (e.g., GP, Retired)	6 (20)
Years worked with men	
1–5 years	11 (37)
6–10 years	6 (20)
11–15 years	3 (10)
16–20 years	3 (10)
20 + years	7 (23)

3.2.1. A whole life approach for deconstructing relationships

Participants confirmed the significant impact that distressed and disrupted relationships had on men's lives and discussed how these transitions heightened men's mental health challenges. In this context, utilizing a whole life approach provided a framework to direct lines of enquiry and encourage personal reflection. This approach required exploring the impact of the distressed and/or disrupted relationship on men's lives (e.g., confidence, friendships, work and/or school), assessing social support and coping strategies, and interpreting men's narratives and associated affective responses to tailor services. For many men, their relationship break-up had far-reaching effects. For example, a 35-year-old Canadian man described how the end of his relationship levered unanticipated challenges manifesting an acute mental health crisis:

So not only was my relationship fragmented, but my friend group, my social network, my housing, my finances, I had just started my masters of social work at that time. So, I was in school, had moved out, my relationship had ended and all these things had happened and all these huge significant life changes in terms of career, school, relationship all within like, weeks...I then ended up taking time off work and yeah, had this panic attack and saw multiple counsellors... probably three or four counsellors.

For this participant and many other men, the relationship break-up seeded significant collateral damage, the sum of which levered men's help-seeking. Many men valued the opportunity to contextualize their help-seeking and inform treatment based on personal anecdotes and life impacts. Related to this, a 48-year-old Australian man suggested that general practitioners (GP) should use men's relationship narratives in the prevention, screening, and triaging of men's mental health challenges:

Maybe every six months' fathers should go to the GP and do a quick, 'alright, let's do a mental health check, how are you managing? How are

Table 2
Characteristics of men (N = 25).

Characteristics	n (%)
Country	
Canada	15 (60)
Australia	10 (40)
Gender	
Man	24 (96)
Non-binary	1 (4)
Age (years)	
20–29	3 (12)
30–39	6 (24)
40–49	11 (44)
50–59	1 (4)
60–69	3 (12)
70+	1 (4)
Sexuality	
Heterosexual	21 (84)
Gay	3 (12)
Bisexual	1 (4)
Education	
Some or all High School	3 (12)
Diploma or Certificate	5 (20)
Some College	3 (12)
Bachelor's Degree	9 (36)
Post Graduate Degree	5 (20)
Employment	
Employed for wages	14 (56)
Self-employed	5 (20)
Looking for employment	1 (4)
Retired	1 (4)
Other	4 (16)
Marital status	
Single, separated or divorced	13 (52)
Partnered or married	7 (28)
Single, never married	5 (20)
Prior relationship duration (years)	
<1	1 (4)
1–5	8 (32)
6–10	7 (28)
11–15	1 (4)
16–20	6 (24)
21–25	1 (4)
26–30	1 (4)
Current living arrangements	
With children	7 (28)
Lives alone	7 (28)
Partner/spouse	4 (16)
Other family member(s) (e.g., parents)	3 (12)
Partner/spouse & child(ren)	2 (8)
Roommates	2 (8)

you coping? How are you dealing with these situations?', and not just an ...anxiety rating thing, an actual kind of scaled version of something where some of the responses might trigger further assessments or further queries.

For health service providers, understanding men's life circumstances was contingent on listening closely to the details of their narratives, rather than simply the endorsement of symptoms. As a 36-year-old Canadian service provider who worked with men in disrupted relationships for 1.5 years explained, men are often indirect with their self-disclosures:

My experience has been that guys are somewhat less likely to say off the hop, 'I've been feeling depressed,' but are more likely to express that in how they talk about their circumstances and how they envision possible futures and things like that.

Health service providers also discussed how men's affect (e.g., anger, guilt, hopelessness) in their narratives offered vital clues to mental health and suicidality risk. Similarly, soliciting details about men's social supports and self-management strategies were key to fully evaluating their treatment needs. As a 36-year-old Australian service provider with 6 years' experience explained:

If there's nothing for the guys to... hold onto, then that's where the

Table 3

Summary of themes and recommendations for working with men in distressed and disrupted intimate partner relationships.

Theme	Summary	Clinical Recommendations
1. A whole life approach for deconstructing relationships	Men's distressed and disrupted relationships shaped and were shaped by broader life challenges that contributed to mental illness and suicide risk. Utilizing a whole life approach involved integrating discussions about wide-ranging issues and injuries to contextualize the distressed and/or disrupted relationship and tailor treatments. Help-seeking men drew a sense of being known and understood by health service providers who used this approach.	- Engage men by inquiring about pre-existing challenges and the impacts of their disrupted relationship on their life (i.e., confidence, friendships, work/school) - Assess men's social support and coping strategies for dealing with their disrupted relationship (i.e., substance use, confidants, exercise) - Interpret men's narratives and affect (i.e., blame, victim, shame, guilt, anger) to identify stress causes and introduce behaviour change
2. Affirming men's relationship emotions and vulnerabilities as normative and changeable	Health service providers' communication was key to men's engagement and trust for sharing emotions and vulnerabilities. Norming vulnerability and emotion states was reliant on health service providers making opportunities for men to express themselves but also strategically deconstructing what was felt as a means for considering the role of masculine norms and need for behaviour change.	- Utilize plain language communication strategies that facilitate men expressing their challenges - Norm introspection and the work of deconstructing what is felt as key to transformative self-improvements - Challenge unproductive gender norms (e.g., men don't cry) and leverage positive masculine roles (e.g., contemporary fatherhood)
3. Tangible 'to do's' in and after a relationship	Offer practical and time-sensitive strategies to satiate a tangible sense of progress. Provide action-oriented 'to do's' (e.g., sleep, journaling, cognitive switching), and cautions about reducing maladaptive behaviours.	- Map and regularly revisit treatment plans and progress over time - Coach men to develop and evaluate actionable strategies in building (and adjusting) their self-management - Schedule follow-up for ongoing work beyond the acute relationship crisis

real problems set in and, you know, you hear the horrible news stories about, they got to a breaking point at that stage and I think that's often because they just think, 'well, there's no point, I've got nothing left, it is completely hopeless', whereas if there's some kind of community around them, preferably with some kind of lived experience that can speak to it, to provide them with the support and that hope for some kind of positive outcomes, some kind of silver lining, then that's what they really need to be encouraged, while going through the really difficult [separation] process.

Predominant here, and in many participants' narratives, were the mental health risks for men's social isolation and loneliness. Noting potential generational differences, a 30-year-old Canadian service provider with 5 years' experience, offered the caveat:

One of the questions we ask in assessment is how is your support network? Do you talk to other people about these sorts of topics? And I

think sort of generally speaking, we're seeing a trend where young men sort of under the age of 25 have a good answer to that question. They'll say something like, yeah, 'I've got a couple of good friends I'll talk to about mental health'.

3.2.2. Affirming men's relationship emotions and vulnerabilities as normative and changeable

Participants discussed the importance of men having opportunities to communicate to sort through their emotional experiences. Suggested herein was that for many men, separation resulted in the loss of an important (and perhaps their only) outlet to communicate openly and honestly. Establishing a therapeutic relationship with the *right* health service provider could serve as a proxy, but also lever men to work toward effectually deconstructing what was felt. A 47-year-old Canadian man explained how his health service provider's prompts enabled him to more fully understand and rationally act on what he was feeling:

He [service provider] didn't give me the scripted counsellor speech of 'you're worth it, you're good'...He was a little bit more hardcore. He would just kind of hit it to me straight, like '[name], you've got to dig deeper'. He started proposing more thought provoking questions...or he would just call me out sometimes...if you're an emotional guy why should you avoid your emotions?...like he would tell it to me straight up.

Health service provider plain language prompts (rather than directives) were valued by many men as an approach that provided agency for how they interpreted and acted on what was felt. This was especially important for men who experienced a profound lack of control with the break-up (i.e., loss of friends, involvement of mediators and/or lawyers). A 30-year-old Canadian service provider with 5 years' experience suggested specific strategies could provide men with permission and persuasion to act differently and potentially disrupt perceived norms:

So sarcasm is a good go-to for men and paradoxical framing. A big one I hear is, '[I was] born this way, I'll die this way. This is how it's always been.' And I love just going down the rabbit hole with the guy, 'yeah, you know, things are terrible. Why are we even bothering? Like why are we even here?'...counsellors should be courageous in being honest about how they approach things. The counsellor says, '...Well, actually crying is a perfectly healthy thing. I found it quite beneficial.' And it breaks the typical model of what they'd expect someone to do.

Highlighted here were health service provider strategies for challenging men to interrogate masculine norms (i.e., crying as synonymous with weakness) as pivots for understanding and authentically expressing their emotions. These reflexive practices also needed to be understood by men as lifelong practices rather than crisis responses. A 28-year-old Canadian man lamented that his couples counsellor did not warn him of the emotional work he needed to do after the relationship ended:

I think that the counsellor could have easily, maybe if she had the experience or she knew our situation better, she could have been like, 'you're about to go through a very rocky time of separation', even though logically it makes sense that you're separating, your emotions are going to be very up and down [but] if she had just warned us about that, that would have been a lot nicer...it's funny because at the beginning of it, the counselling helped a lot, but then counselling didn't help so much at the end of it.

Evident here was the man's surprise that his emotional work would be ongoing, and that the distress in the relationship would not be eased by the break-up. Within this context, participants pointed to the value for appealing to men's protector and provider roles. For example, contemporary fathering identities could be used to frame men's collaborative concessions as selfless and strength-based. A 36-year-old Australian service provider with 6 years' experience explained:

That role as a father was a particularly powerful leverage point and I think there would be potential to use that in a range of different contexts tapping into that identity as a father, 'what is your role as a father?' You know, everyone says that they want to be a good father. So how can that be leveraged and utilized to really encourage people to invest in making

sure that they develop the most positive relationships they can and contribute positively to family functioning in that way.

3.2.3. *Tangible 'to do's' during and after a relationship break-up*

Participants discussed how health service providers can support men through developing tangible 'to do's'. In circumstances where men had engaged in maladaptive behaviours (i.e., substance use) in response to their circumstances (e.g., loss of housing), the practicalities for addressing those challenges in tandem with managing the psychological distress were key. A 36-year-old Canadian service provider with 1.5 years' experience reflected on the limits inherent to being taught to focus on and address men's deficits:

I did supervision with my clinical supervisor and speaking about a gentleman I'm serving and my supervisor was asking me about some of these basic needs questions and really challenged me and said, 'you know, it sounds like this guy's in survival mode and you're focused on your agenda of anger and family violence and that's important, but isn't it kind of dehumanizing that you're not making a space to help this dude with his basic needs', and I spent 10 years working in homelessness.

Referenced here were health equity orientations for working with men who were living in marginalizing conditions. In essence, the context and capacity for men to proactively act needed to be fully seen by health service providers to fairly evaluate (and mobilize) the men's potential. For many men, tools were needed to manage their vulnerabilities and emotions. Herewith, reducing maladaptive behaviours (e.g., substance misuse) and affirming the development of sustainable positive coping strategies (e.g., talking to trusted others about mental health challenges) was a focus. This included stress-management (e.g., meditation, mindfulness, diaphragmatic breathing) and mood control (e.g., emotional awareness, regulation, and anger management). A 30-year-old Canadian service provider with 5 years' experience elaborated:

So first we deal with the acute stress, the guy's having panic attacks or he's drinking and is at risk of self-harm or [other] risky behaviour... we give them education, a concrete tool to take home for the first couple of weeks. Then there's usually some sort of self-compassion process or change in the way they think, feel, and function. And then we look at kind of tweaking that process and creating a new pattern of existing in the world. So starting to talk to their friends about mental health or their emotions, starting to open up to their wives, starting to kind of use some self-care, some tools to manage stress. Then we keep maintaining those and tweaking them and applying them to different circumstances so they can continue to function and they leave our clinic and use the tools and we do a six-week check-in and we say, 'okay, so how did that six weeks go?' And then either it went well, or it went badly. And so more sessions are needed or they just kind of have a tune-up and they go on their way.

Exemplified here were tailored ongoing approaches based on men's progress with tangible behavioural 'to do's'. Highlighted also was the importance of providing regular and meaningful follow-up to track progress and make adjustments. Many men worked to make meaningful progress and recognized their relationship breakdown as an opportunity for personal growth. A 33-year-old Canadian man confirmed the importance of having an action plan that he could follow and maintain over time:

He [therapist] mapped out my life for me as part of the session and tried to put everything in perspective. That was the first session. The second session was strategies and mapping out going forward, and then I actually left with the map. That is something concrete rather than just, 'oh another session where you talked for an hour' and yeah, like you're venting and it's good because it's cathartic and you leave with maybe a few little strategies. I think having it down on paper, maybe it just helps for me visually, and something concrete, you know, not just talk.

The reference to actionable strategies was consistent in the men's and health service providers' interviews, wherein the work of transitioning through a break-up was best served by planned and thoughtful actions that could be evaluated. The tangibility of 'to do's' underpinned men's satisfaction with the help, and this approach tapped the

rationality and purpose most men craved in the aftermath of the break-up.

4. Discussion and conclusion

4.1. Discussion

The aim of this study was to describe key considerations for working with men in the context of distressed and disrupted intimate partner relationships. While cultures vary (e.g., [27]), Canada and Australia have comparable health care systems and intimate partner relationship conventions, and the findings did not significantly differ by location. The findings confirm the connectedness of men's intimate partner relationships and their mental health [7] and the important role of professional services therein [28]. In these contexts, men's increased risk for mental illness and suicide demand tailored services and delivery that utilize a whole life approach for deconstructing relationships, affirm men's relationship emotions and vulnerabilities as normative and changeable, and offer tangible 'to do's' in and after a relationship. Given the potential for men's low engagement and dropout from professional services [29], the current study findings offer poignant and perhaps imperative guidance for gendered approaches to men's mental health in the context of distressed and disrupted intimate partner relationships – and more broadly. While invariably connected, themes are discussed herein as discrete to position and contextualize study contributions amid the broader men's mental health and intimate partner relationship research.

Regarding *A whole life approach for deconstructing relationships*, these thematic findings underscore the need for health service providers to contextually understand the pain points implicated for men's distressed and/or disrupted intimate partner relationships. Herein, men's challenges should be interpreted through what is shared directly, but also in how men depict themselves within their relationship narratives. As Oliffe et al. [30] suggest, men's break-up narratives afford vital clues to the specificities and salencies of their issues and injuries – and by extension, the treatment strategies for addressing those injuries. Health service providers must be attuned to the gendered manifestations of mental ill health in men's experiential accounts. For example, men's relationship narratives may signal a range of distress symptoms such as risk taking, aggression, and/or substance use [31–33]. Considering men's behaviour as a product of distress, rather than the root cause, challenges deficit-based depictions of men and masculinities and may be a critical step to developing therapeutic alliances that engage men [34, 35].

The *Affirming men's relationship emotions and vulnerabilities as normative and changeable* findings support previous research indicating the value of working with men to deconstruct and gain insight from their feelings [18]. In light of difficulties with identifying and expressing emotional distress [36], health service providers can support men in understanding and navigating what might otherwise be denied and/or concealed. There is of course some untangling of normative masculinities, whereby stoicism and emotional restraint need to be eased [7]. Qualitative accounts of men's health-related beliefs and practices continue to demonstrate how masculinities can be adapted or renegotiated in transformative ways [37,38]. In the context of relationship break-ups, the need to sit with what might be uncomfortably experienced (i.e., guilt, shame, regret [11]) may be framed as demanding emotional strength and hard work ethic for introspection as a means to securing health gains. The ability to position, lever, and contextualize masculinities requires creative and careful work to meaningfully reframe and transform masculine norms and roles. Emergent is gender-based training for health service providers (e.g., Men in Mind [39]), whereby health service providers reflect on their own socialized assumptions and develop skills in working with men and masculinities.

The *Tangible 'to do's' in and after a relationship* findings add to a growing body of work which highlights some men's preference for

action-oriented strategies for mental health promotion [40,41]. In the context of relationships, strategies such as relaxation and stress reduction techniques (meditation, mindfulness) have emerged as feasible in men's mental health when brokered as positive enactments of masculinity [42]. Herein, men's 'to do's' involved strength-based tasks that are actionable and measurable wherein the drivers are health benefits – rather than illness consequences. Talk therapy also prevails in this action orientation – and as previously highlighted, dynamism language is key to men's introspective work [43] along with the centrality of mapping strategies and progress in talk therapies [36]. Highlighted herein is the importance of men's agency and health service providers' role in subtly guiding skill building and action plans to lever and map deeper therapeutic work.

This study was limited by the cross-sectional design and future research is needed to examine the longitudinal impacts of gendered approaches on men's relationship work, and avenues for attracting men to professional care services. Additionally, the present study focused on identifying overarching principles for working with men in distressed and disrupted relationships, which should not obscure unique variations and needs of men in diverse transitions phases, situations and contexts. The findings may also vary by specific help-service context.

4.2. Conclusion

Men's elevated risk for mental illness in distressed and disrupted relationships requires gender transformative approaches that consider broader life circumstances, vulnerabilities and emotions and tangible to do's to address those challenges. Strategic assessments informing tailored treatments will likely sustainably engage men who experience distressed and disrupted intimate partner relationships to reduce their mental illness and suicide risks.

4.3. Practice implications

Increasing numbers of men are seeking professional mental health care [10], and it is imperative that health service providers recognize and treat men in gender-transformative ways. This study provides key insights for strategically working with men to reduce the formidable mental health risks associated with distressed and disrupted intimate partner relationships. The need to upskill clinicians is omnipresent, as are emergent training programs, including Men in Mind [44], to support health service providers in their work with men.

CRediT authorship contribution statement

Sharp: Conceptualization, Formal analysis, Data curation, Writing – original draft, Writing – review & editing, Visualization. **Ogrodniczuk:** Supervision, Writing – Original Draft, Writing – Review & Editing, Funding acquisition. **Sha:** Investigation, Writing – Review & Editing. **Kelly:** Investigation, Data Curation, Writing – Review & Editing. **Montaner:** Investigation, Writing – Original Draft, Project administration. **Kealy:** Writing – Review & Editing, Funding acquisition. **Seilder:** Writing – Review & Editing, Funding acquisition. **Rice:** Writing – Review & Editing, Funding acquisition. **Oliffe:** Conceptualization, Methodology, Investigation, Supervision, Writing – Original Draft, Writing – Review & Editing, Funding acquisition.

Declaration of Competing Interest

The authors declare that they have no known competing financial interests or personal relationships that could have appeared to influence the work reported in this paper.

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Interview guide – health service providers

Interview questions

- Tell me a little bit about yourself – e.g., what is your position, what type of work do you do, what is the mandate of your agency and/or goal of your services?
- When and how do men typically find their way to relationship services?
 - o What prevention or relationship building services exist for men?
- What are men's needs during a relationship breakdown?
 - o What services do they access?
 - o (prompt mandated, mental health, anger management)
 - o What is key to the success of these services for men?
 - o What are the mental health needs of men who are in a troubled relationship?
 - o What aspects of services work especially well for men?
 - o What are the key ingredients for engaging and retaining men in 'relationship' services?
 - o What works especially well for many men? What does not work for men?
- How do you support men to build relationships and navigate breakdowns?
- After a relationship breakdown what services do men typically use?
 - o When and how do men typically reach out for help?
 - o What are the pathways for men to access these services?
 - o What are the facilitators and barriers to providing services to men in the aftermath of a partnership breakdown?

Overall

- What is the catalyst for most men seeking help for their relationships?
- What do you understand as the main barriers for men accessing and sustaining engagement with services?
 - o What kind of help or skills do men usually think they need in the beginning?
 - o How does that change as men engage in a relationship building program?
- What works best for men in terms of accessing and sustaining engagement with services?
 - o What is an example of a success story in terms of working with a man through a relationship breakdown?
 - o What advice do you have for others working with men in the 'relationship' space?
 - o What strategies do you/providers employ to indicate their services are for men?
 - o What mechanisms and means are used to engage men when they access services?
- What works for men in terms of their masculinity in relationship breakdowns and seeking help?
- What aspects of masculinity fail men when they experience a relationship breakdown?

- Any other thoughts or insights you'd like to share with us?

Interview guide – help-seeking men

Interview questions

- Tell me a little bit about yourself – e.g., what is your background, employment and relationship status?
- After the relationship ended what were your thoughts about and strategies for moving on?
 - o What was most helpful in the aftermath of the relationship ending?
 - o What was most unhelpful in trying to deal with the relationship ending?
 - o What assistance did you get to move on from the relationship?
 - What helped and what would have helped to move through that?
 - o What do you wish you'd done differently after the relationship ended?
 - o What were your stress levels like?
 - o How did you self-manage what you were feeling?
 - o How were the other relationships in your life? (family, work, friends, new partner)
 - o What did you learn and take forward from the experience?
- What was the collateral damage from the relationship breakdown?
- What is your advice for other guys who are in the aftermath of a relationship breakdown?
- When should men access help for building better relationships?
 - o What are the best forms of help that men can access?
 - o Which aspect of relationships are most important to work on or improve?
 - o What are some effective program/intervention modalities for supporting men before/during/after relationship breakdown?
 - o What should men avoid in the context of a troubled relationship and/or breakdown?
 - o Drawing on personal experience, what recommendations can you offer for future program implementors?
- What were your experiences with receiving professional help?
- Which aspects of the help you received were most valuable?
- Any other thoughts or insights you'd like to share with us?

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